

Fraud Risk & Investigation Guidelines

a. Known Fraud Patterns

- Phantom Billing: Submitting claim statements for medical services or tests never rendered.
- Upcoding: Submitting higher-severity procedure codes than the service actually rendered.
- Provider Collusion: Abnormally high claim submission volumes concentrated within a single provider ID.
- Rapid Claim Spikes: Multiple high-value claims submitted within 30 days of policy creation.
- Unbundling: Splitting a single procedure into multiple claim components to maximize payouts.

b. Investigation Procedures

- Step 1: Extract model prediction, SHAP risk drivers, and peer group deviation ratios.
- Step 2: Cross-reference provider ID against historical watch-lists and volume percentiles.
- Step 3: Validate billing documentation against policy coverage limits and itemized receipts.
- Step 4: Issue formal Document Request or refer case to Special Investigation Unit (SIU).

c. Escalation Criteria

- Claims where the claim amount exceeds peer average by more than \$10,000 or >200% deviation.
- Policyholders with 2 or more rejected or denied claims within the preceding 12 months.
- Claims associated with providers whose historical fraud rate exceeds 15%.